

FACE SYMPTOMS

Give urgent attention to the patient with face symptoms and any of:

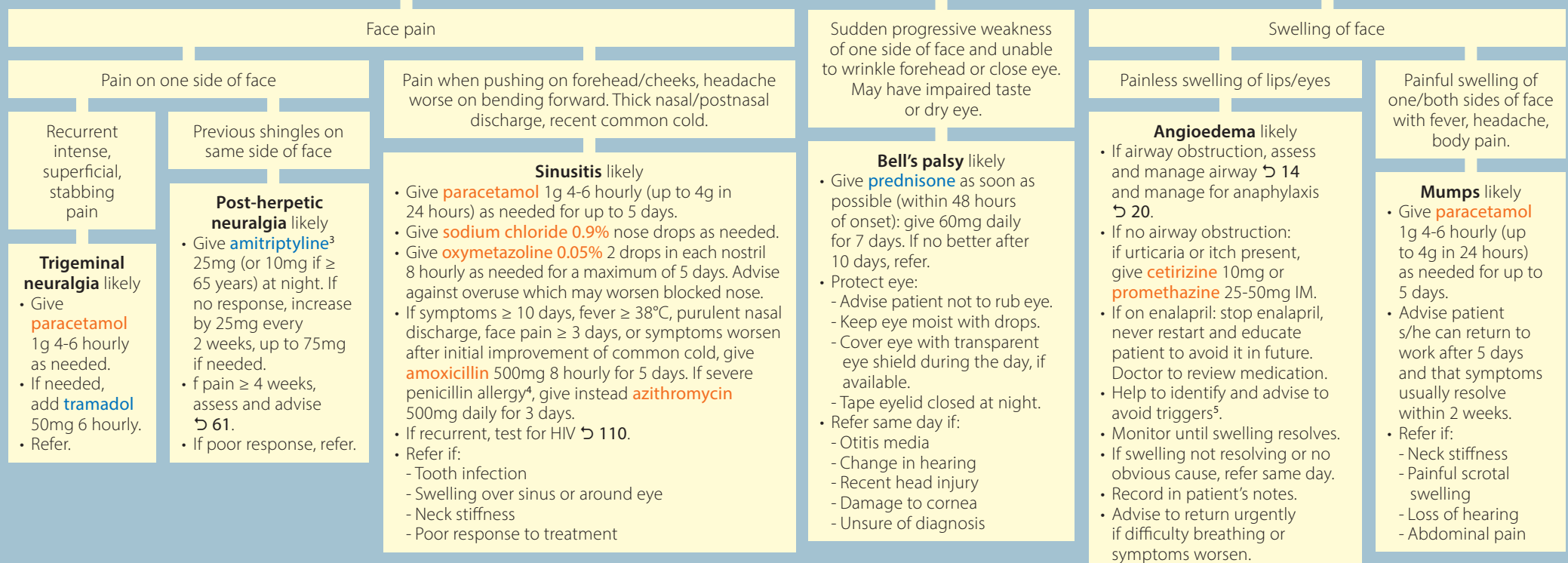
- New sudden asymmetric weakness or numbness of face (with no/minimal forehead involvement), arm or leg; difficulty speaking or visual disturbance: consider **stroke** or **TIA** → 136.
- Sudden face/tongue swelling and any of: difficulty breathing, BP < 90/60, dizziness/collapse, abdominal pain, vomiting or exposure to possible allergen¹, check for anaphylaxis → 20.
- Painful red facial swelling and temperature ≥ 38°C: **facial cellulitis** likely
- New swelling of face and blood/protein in urine: **kidney disease** likely

Manage and refer urgently:

- If likely facial cellulitis with if whole eyelid swollen, red and painful, **orbital cellulitis** likely: give **ceftriaxone** 2g IV²/IM. Avoid injecting > 1g IM at one injection site.
- If **kidney disease** likely: if pulse > 100 or respiratory rate > 30, give face mask oxygen and **furosemide** 80mg slow IV, avoid IV fluids. If BP > 150/100, give **amlodipine** 5mg and **furosemide** 40mg orally.

Approach to patient with face symptoms not needing urgent attention

- If rash on face → 67.
- If gum or tooth problem → 36.
- Manage according to face symptom/s:



¹Common allergens include medication, food or insect bite/sting within the past few hours. ²Do not mix Ringer's lactate and IV ceftriaxone. Flush IV line with **sodium chloride 0.9%** before and after IV ceftriaxone. ³Avoid if on bedaquiline. ⁴History of anaphylaxis, urticaria or angioedema. ⁵Common triggers include foods (milk, eggs, nuts, wheat, seafood), medications, insect bites/stings and latex.